NCE EXAM PREP

Practice Exam 09

Domain 3

Treatment Planning & Continuity of Care

26 Practice Questions | 34 minutes

A, B, C answer choices

Answer Key and Explanations included

> **Instructions.** Select the single best answer for each question. Record your answers separately. This domain practice set is drawn from Full-Length Practice Exam 02. Use the suggested 34-minute practice timer. Do not consult Part B until you complete Part A. These domain timings are editorial practice allocations, not official NBCC section limits.

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PART A — QUESTIONS

Choose one answer (A, B, or C) per question.

Question 1

> An adult in rehabilitation counseling after a stroke uses supported communication because of mild expressive aphasia. The client's spouse wants treatment to focus on returning to work, but the client consistently identifies independent bus travel as the immediate goal.

Which initial planning response best supports collaboration?

A) Establish an accessible transit goal with the client and schedule later review of the work concern

B) Adopt return to work as the first goal because the spouse can describe daily functioning more easily

C) Postpone goal setting until the client can communicate without supported methods

Question 2

How are client strengths used most effectively in a treatment plan?

A) By documenting positive qualities separately from goals so they do not influence clinical decisions

B) By translating demonstrated abilities and available supports into resources for attaining objectives

C) By treating strengths as evidence that counseling barriers will resolve without specific planning

Question 3

> A client diagnosed with persistent depressive disorder says, “Persistent must mean permanent, so goals will not matter.” The client is considering ending counseling before the initial plan is completed.

How should the counselor address the diagnosis in treatment planning?

A) Avoid the term in future sessions so the client does not become discouraged by diagnostic language

B) Emphasize that the diagnosis is correct and proceed with goals even if the client remains doubtful

C) Clarify what the diagnosis describes, correct the permanence assumption, and invite questions while setting goals together

Question 4

> A 7-year-old receiving counseling after the death of a grandparent has emerging literacy skills. The current plan asks the child to write three paragraphs about emotions each week, but the child engages readily through drawing and storytelling.

Which revision best fits the child's developmental needs?

A) Replace the writing task with drawing or feeling choices followed by a brief child-led story

B) Keep the writing task so the child learns to express grief in an age-independent format

C) Ask the caregiver to write the paragraphs because adult language is more clinically useful

Question 5

A client accepts a referral to a pain-management specialist but declines to authorize communication between providers. The client still wants help preparing for the consultation and understands that the counselor cannot coordinate directly without permission.

Which response best supports care continuity while respecting the client's decision?

A) Contact the specialist with only diagnostic information because limited coordination does not require permission

B) Provide referral details and help the client prepare a concise summary the client may choose to share

C) Withdraw the referral because continuity is possible only when providers communicate with one another

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Question 6

A client completing treatment for gambling disorder identifies the start of professional football season as a period of increased urges. The client already uses voluntary self-exclusion, bank transaction blocks, and weekly financial accountability with a trusted partner.

Which maintenance plan best strengthens relapse prevention?

A) Remove the transaction blocks gradually to determine whether the client can now gamble in moderation

B) Continue the safeguards and define a same-day support and treatment contact plan for strong urges or any bet

C) Rely on the partner to detect gambling activity before deciding whether additional support is necessary

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Question 7

Which information should have the greatest influence when selecting a counseling modality for a treatment plan?

A) The client's needs, preferences, access, relevant evidence, and any contraindications

B) The modality most commonly paired with the client's diagnostic label in agency templates

C) The approach the counselor uses most often, regardless of the client's circumstances

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Question 8

In family counseling, two caregivers want their 16-year-old to stop all online gaming. The adolescent wants fewer arguments and says gaming is the main way of maintaining friendships. All agree that meals have become a frequent conflict point.

Which initial goal best reflects collaborative prioritization?

A) Establish a shared trial of device-free meals while continuing to assess gaming, friendships, and family conflict

B) Adopt complete gaming abstinence because the caregivers hold responsibility for household rules

C) Focus only on the adolescent's friendship needs because treatment goals must belong to the youngest family member

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Question 9

> A transgender client seeks counseling for anxiety and says they are undecided about any social transition. A draft plan written by a previous provider lists beginning a social transition as a treatment goal, but the client does not identify that as an objective.

What is the most appropriate plan revision?

A) Retain the transition goal because it may eventually reduce anxiety even if the client is uncertain

B) Remove the presumed goal and collaborate on the client's anxiety priorities, exploring identity only as the client wishes

C) Replace the transition goal with an objective to decide on a transition before anxiety treatment begins

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Question 10

Which use of case conceptualization contributes most directly to treatment planning?

A) Compiling a complete chronological history without forming conclusions about what maintains the concern

B) Selecting interventions from the diagnostic label without considering current protective factors

C) Organizing contributing and protective factors into testable hypotheses that guide goals and interventions

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Question 11

> Two partners identify “communicate better” as a counseling goal. They currently have about five escalating arguments each week and agree that a short evening check-in is feasible.

Which objective best makes the goal measurable and attainable?

A) The partners will eliminate all disagreements during daily check-ins for four weeks

B) The partners will complete a 10-minute check-in four evenings weekly and track escalation for four weeks

C) The partners will hold check-ins when needed and discuss progress at the next four sessions

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Question 12

Which outcome best demonstrates that a referral loop has been closed?

A) The receiving service has the needed authorized information, responsibilities are clear, and the client has connected with care

B) The counselor has sent a referral form and documented the date, even though receipt has not been confirmed

C) The client has received several provider names and understands that arranging care is now solely the client's responsibility

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Question 13

> An adult in supported living repeatedly uses the staff call button when routine check-in times are unpredictable. The client can use a picture schedule and can make a direct request for attention when prompted. Staff report that immediate attention after repeated calls has unintentionally reinforced the pattern.

Which behavioral management plan best fits these facts?

A) Schedule predictable check-ins, teach the direct request, reinforce its use, and track call-button frequency

B) Remove the call button for increasing periods after repeated use and record whether the client complains

C) Ask each staff member to respond according to personal judgment so the client learns flexibility

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Question 14

> A telehealth platform disconnects during a session with a client who has episodic panic attacks but no current acute safety concern. The counselor and client previously documented a backup telephone number, the client's current location, and local emergency options.

What should the counselor do to implement the disruption plan?

A) Use the agreed backup number, confirm the client's status, and decide whether to resume or reschedule

B) Wait for the client to reconnect so that the counselor does not cross a communication boundary

C) Contact emergency services immediately because any disconnection during mental health care is an emergency

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Question 15

Which feature makes a community referral most actionable for a client?

A) Current eligibility and availability are verified, the service fits the client's needs and preferences, and access steps are clear

B) The client receives a large directory containing every agency that has offered a related service in recent years

C) The counselor provides the name of a well-known agency and asks the client to investigate whether it is appropriate

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Question 16

> A client's insurance coverage will end in two weeks after a job change. The client is stable but needs ongoing counseling and agrees to transfer to a sliding-scale community clinic. The current counselor can provide two bridge sessions under agency policy.

Which plan best addresses the financial transition?

A) Use the final sessions to process the insurance loss and let the client contact clinics after coverage ends

B) Discharge now so the client can use the remaining coverage to pay the new clinic independently

C) Confirm an affordable intake, coordinate authorized information, and use bridge sessions and contingency resources as needed

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Question 17

A college counseling center can provide only two more sessions to a student who needs longer-term trauma treatment. The student accepts a referral, but the receiving clinic's first appointment is six weeks away. Center policy permits brief bridge contacts during a documented transition.

Which plan best supports continuity during the wait?

A) Use both remaining sessions now and tell the student to seek emergency care if distress increases later

B) Ask the student to contact the receiving clinic independently and return after the six-week wait if needed

C) Confirm the referral, schedule permitted bridge contacts, and document coping and urgent-care options for the interim

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Question 18

A client with a traumatic brain injury has slowed information processing and difficulty retaining lengthy verbal instructions. A standard 90-minute, fast-paced counseling group is available, but the client prefers individual work and benefits from summaries and repetition.

Which modality plan is most responsive to the available information?

A) Enroll the client in the standard group and require detailed note-taking after each topic

B) Postpone counseling until processing speed and memory return to their previous levels

C) Use shorter individual sessions with paced content, accessible summaries, and checks for understanding

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Question 19

A client whose apartment is heavily cluttered identifies inviting grandchildren to visit as a meaningful long-term goal. A whole-home cleanout feels overwhelming, but the client agrees that the blocked exit path is the most urgent area and believes one small section can be addressed at a time.

Which initial objective is most appropriate?

A) The client will clear the entire apartment with family help during the next available weekend

B) The client will clear and maintain one agreed section of the exit path within two weeks, then review the next step

C) The client will postpone all home-based objectives until the idea of a complete cleanout no longer causes distress

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Question 20

A client considering whether to leave a family-owned business says that a trusted aunt has always helped with major decisions. The client wants the aunt's perspective included but does not want the aunt to decide the counseling goals.

How should the counselor reflect this preference in the plan?

A) Exclude the aunt because involving relatives prevents independent client decision-making

B) With the client's permission, define a limited consultative role for the aunt while keeping goals client-directed

C) Make family agreement the outcome criterion because the decision affects a family-owned business

Question 21

A client receiving counseling for panic symptoms is newly diagnosed with diabetes. The medical provider explains that several episodes of sweating, shaking, and confusion were caused by low blood glucose, although the client also continues to experience panic attacks in other situations. The client consents to care coordination.

How should the counselor revise the treatment plan?

A) Remove all anxiety goals because the physical symptoms now have a medical explanation

B) Continue the original plan unchanged because medical information is outside counseling treatment planning

C) Coordinate relevant care and help the client distinguish episodes while retaining appropriate panic-related goals

Question 22

A client with opioid use disorder is preparing to leave residential treatment. The client plans to continue prescribed buprenorphine, has a supportive sister, and wants outpatient counseling near home. Reduced opioid tolerance increases overdose risk after discharge.

Which aftercare plan is most appropriate?

A) Confirm medication and counseling appointments, include overdose-prevention resources, and define support and reentry steps

B) End buprenorphine at discharge so the counselor can evaluate recovery without medication

C) Give the client general program numbers and wait for the client to request help after returning home

Question 23

A community agency requires outreach after two missed appointments and closes a case after 30 days without client contact. Twenty days after the last visit, a client calls, explains that phone service was disconnected, and asks to resume counseling. The client reports no acute safety concern.

Which response best supports continuity within the stated policy?

A) Close the case because two missed appointments demonstrate that the client is not ready to participate

B) Review the access barrier, update backup contact and attendance plans, and reschedule while the case remains open

C) Keep the case open indefinitely without revisiting communication plans because the client has now made contact

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Question 24

A treatment team determines that weekly outpatient counseling is no longer sufficient for a client with rapidly escalating manic symptoms, severe sleep loss, and major functional impairment. The client agrees to an urgent evaluation for a higher level of care.

What should the outpatient counselor do to support continuity?

A) Increase between-session assignments while waiting until the next routine appointment to review symptoms

B) End outpatient services immediately so the higher-level program assumes responsibility without overlap

C) Coordinate the urgent evaluation, transfer authorized information, and maintain a bridge until care is accepted

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Question 25

A community college student seeks help for test anxiety and avoidance. The student works two jobs and cannot afford weekly private counseling, but an evening campus skills group is free, and a trusted classmate already supports study routines.

Which plan best integrates the student's strengths and barriers?

A) Recommend weekly private counseling because individual treatment permits the most personalized plan

B) Delay treatment until the student can reduce work hours and attend daytime appointments consistently

C) Use the accessible evening group and, with the student's agreement, incorporate peer accountability into objectives

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Question 26

> An adult leaving a county behavioral health diversion program needs ongoing counseling, stable housing, and transportation to appointments. The client consents to coordination and has chosen a community clinic but has not yet contacted the housing or transportation programs.

Which transition plan best reduces the likelihood of a service gap?

A) Give the client a printed resource list and close the case on the scheduled program end date

B) Confirm appointments and eligibility with the selected services, clarify responsibilities, and address transportation before discharge

C) Keep the client enrolled in the diversion program until housing and transportation problems are permanently resolved

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PART B — ANSWER KEY AND EXPLANATIONS

Answers match the numbered items in Part A.

Question 1 — Correct: A

Correct Answer: Establish an accessible transit goal with the client and schedule later review of the work concern

Explanation: The client has communicated a consistent immediate priority, and supported communication is a valid means of collaborative participation. Addressing transit first respects autonomy while leaving the spouse's concern available for later review rather than allowing it to replace the client's goal.

Question 2 — Correct: B

Correct Answer: By translating demonstrated abilities and available supports into resources for attaining objectives

Explanation: Strengths become clinically useful when they are linked to specific goals, barriers, and action steps. Listing them separately or assuming they eliminate the need for planning does not show how they will support change.

Question 3 — Correct: C

Correct Answer: Clarify what the diagnosis describes, correct the permanence assumption, and invite questions while setting goals together

Explanation: The client's misunderstanding is directly affecting willingness to plan treatment. Clear, collaborative review can distinguish a diagnostic course descriptor from a prediction that change is impossible and can restore informed participation.

Question 4 — Correct: A

Correct Answer: Replace the writing task with drawing or feeling choices followed by a brief child-led story

Explanation: The revision preserves emotional expression while matching the child's literacy level and demonstrated ways of engaging. Retaining an inaccessible task or substituting the caregiver's language would reduce the child's direct participation in treatment.

Question 5 — Correct: B

Correct Answer: Provide referral details and help the client prepare a concise summary the client may choose to share

Explanation: The client may pursue the referral without authorizing direct provider communication. Supporting client-mediated information sharing preserves autonomy and continuity, whereas contacting the specialist anyway would disregard the client's stated decision.

Question 6 — Correct: B

Correct Answer: Continue the safeguards and define a same-day support and treatment contact plan for strong urges or any bet

Explanation: Existing environmental safeguards reduce access to gambling, and a rapid response plan limits escalation if risk rises or a lapse occurs. Removing protections to test moderation increases exposure to a known seasonal trigger, while relying only on a partner delays the client's own response.

Question 7 — Correct: A

Correct Answer: The client's needs, preferences, access, relevant evidence, and any contraindications

Explanation: Modality selection should integrate clinical fit with client preference, feasibility, evidence, and safety. A diagnostic label or counselor habit alone cannot establish that a particular modality is appropriate for an individual client.

Question 8 — Correct: A

Correct Answer: Establish a shared trial of device-free meals while continuing to assess gaming, friendships, and family conflict

Explanation: The proposed trial begins with a concrete area of agreement and allows the family to gather information about the wider concerns. Adopting either the caregivers' total abstinence demand or only the adolescent's perspective would bypass collaborative family goal setting.

Question 9 — Correct: B

Correct Answer: Remove the presumed goal and collaborate on the client's anxiety priorities, exploring identity only as the client wishes

Explanation: Treatment goals should reflect the client's stated needs rather than an assumed course of gender expression. The counselor can remain open to identity exploration without making a transition decision a prerequisite for anxiety care.

Question 10 — Correct: C

Correct Answer: Organizing contributing and protective factors into testable hypotheses that guide goals and interventions

Explanation: A useful conceptualization explains how relevant factors may contribute to or protect against the current problem and connects those hypotheses to the plan. A chronology or diagnosis without such integration provides less individualized direction.

Question 11 — Correct: B

Correct Answer: The partners will complete a 10-minute check-in four evenings weekly and track escalation for four weeks

Explanation: This objective specifies an observable shared behavior, frequency, duration, monitoring method, and time frame that the partners consider feasible. Eliminating all disagreement is unrealistic, whereas holding check-ins only “when needed” is insufficiently measurable.

Question 12 — Correct: A

Correct Answer: The receiving service has the needed authorized information, responsibilities are clear, and the client has connected with care

Explanation: A closed-loop referral verifies that the intended connection occurred and that essential information and roles are in place. Sending paperwork or offering names documents an attempt but does not demonstrate successful linkage.

Question 13 — Correct: A

Correct Answer: Schedule predictable check-ins, teach the direct request, reinforce its use, and track call-button frequency

Explanation: The plan addresses the apparent function of the behavior by increasing predictability and teaching a replacement request, then measures change. Removing the call button may restrict access to needed help, while inconsistent staff responses would continue unpredictability.

Question 14 — Correct: A

Correct Answer: Use the agreed backup number, confirm the client's status, and decide whether to resume or reschedule

Explanation: The documented backup arrangement exists to guide a predictable technology disruption. A disconnection alone does not establish an emergency, while simply waiting would fail to use the continuity plan both parties established.

Question 15 — Correct: A

Correct Answer: Current eligibility and availability are verified, the service fits the client's needs and preferences, and access steps are clear

Explanation: An actionable referral is current, appropriate, and specific enough for the client to use. A broad directory or an unverified agency name shifts the burden of determining basic fit and availability to the client.

Question 16 — Correct: C

Correct Answer: Confirm an affordable intake, coordinate authorized information, and use bridge sessions and contingency resources as needed

Explanation: The foreseeable loss of coverage calls for confirmed affordable follow-up and a plan for the interval between providers. Processing the loss alone or discharging early would not establish ongoing access to care.

Question 17 — Correct: C

Correct Answer: Confirm the referral, schedule permitted bridge contacts, and document coping and urgent-care options for the interim

Explanation: The six-week wait creates a foreseeable gap that can be reduced through confirmed linkage, available bridge care, and a clear interim plan. Using the remaining sessions immediately or relying on the student to manage the transition alone does not address the waiting period.

Question 18 — Correct: C

Correct Answer: Use shorter individual sessions with paced content, accessible summaries, and checks for understanding

Explanation: The selected format responds to documented processing and memory needs as well as the client's preference. Requiring an unmodified fast-paced group or postponing all care would create unnecessary barriers rather than adapting the plan.

Question 19 — Correct: B

Correct Answer: The client will clear and maintain one agreed section of the exit path within two weeks, then review the next step

Explanation: The objective is specific, safety-relevant, time-limited, and scaled to the client's current readiness while serving the meaningful long-term goal. A rapid whole-home cleanout is not presented as attainable, and postponing all action overlooks the client's willingness to begin gradually.

Question 20 — Correct: B

Correct Answer: With the client's permission, define a limited consultative role for the aunt while keeping goals client-directed

Explanation: The plan can honor the client's relational decision-making preference without transferring ownership of treatment goals. Automatically excluding family or requiring family agreement would both disregard the role the client explicitly requested.

Question 21 — Correct: C

Correct Answer: Coordinate relevant care and help the client distinguish episodes while retaining appropriate panic-related goals

Explanation: The new diagnosis changes the interpretation of some episodes but does not account for every panic attack described. Coordinated revision helps the client respond appropriately to both medical and anxiety-related symptoms without discarding relevant goals or ignoring new information.

Question 22 — Correct: A

Correct Answer: Confirm medication and counseling appointments, include overdose-prevention resources, and define support and reentry steps

Explanation: Confirmed follow-up, continuity of prescribed medication, overdose-prevention planning, and clear supports address the high-risk period after residential discharge. Abruptly stopping medication or providing only general numbers would weaken rather than strengthen continuity.

Question 23 — Correct: B

Correct Answer: Review the access barrier, update backup contact and attendance plans, and reschedule while the case remains open

Explanation: The client reestablished contact before the stated closure point and identified a correctable access barrier. Revising the communication and attendance plan supports reengagement within policy, whereas automatic closure or indefinite inaction would not address continuity.

Question 24 — Correct: C

Correct Answer: Coordinate the urgent evaluation, transfer authorized information, and maintain a bridge until care is accepted

Explanation: The team has determined that the current level is inadequate, so timely coordination and a defined bridge are needed until the receiving service accepts care. Routine homework is insufficient, and immediate disengagement could create a dangerous gap.

Question 25 — Correct: C

Correct Answer: Use the accessible evening group and, with the student's agreement, incorporate peer accountability into objectives

Explanation: This plan uses a feasible no-cost service and an existing support while preserving the student's control over peer involvement. Recommending unaffordable care or waiting for work circumstances to change would leave identified barriers unaddressed.

Question 26 — Correct: B

Correct Answer: Confirm appointments and eligibility with the selected services, clarify responsibilities, and address transportation before discharge

Explanation: Confirmed access and clearly assigned next steps make the transition more reliable than a resource list alone. Keeping the client in a time-limited program until every social problem is permanently resolved is neither necessary nor realistic.
